

AFib Anticoagulation Restart Timing After Acute Ischemic Stroke

Atrial Fibrillation Post-Stroke Anticoagulation Resumption Protocol & CATALYST Timing

CLINICAL EFFICACY & SAFETY

RCT and individual-patient meta-analysis data support early DOAC initiation in carefully selected AFib-related ischemic stroke patients, especially mild-to-moderate infarcts without high-risk hemorrhagic transformation. Early treatment has not shown excess symptomatic intracranial hemorrhage (sICH) versus delayed treatment and may reduce recurrent ischemic stroke; DOACs are preferred over warfarin for most nonvalvular AF patients when anticoagulation is indicated.

1. Stroke Severity Classification (ELAN Imaging Criteria)

Minor / Small Infarct

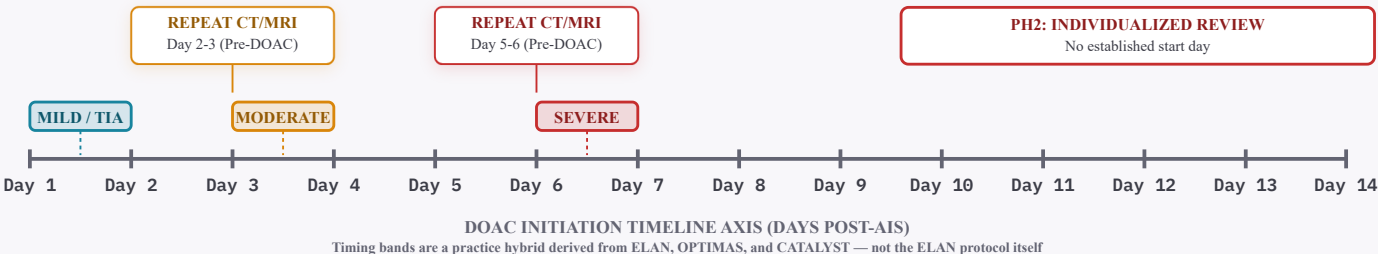
- TIA or infarct ≤ 1.5 cm on brain imaging. NIHSS can guide bedside risk but is not the ELAN definition.

Moderate Infarct

- Cortical superficial-branch lesion, internal border-zone lesion, or deep-branch lesion > 1.5 cm.

Major / Large Infarct

- Complete vascular territory, ≥ 2 moderate lesions, large multilobar infarct, or brainstem/cerebellar lesion ≥ 1.5 cm.



2. Bedside DOAC Dosing & Adjustment Guide

Drug	Standard Dose	Dose Reduction Criteria
Apixaban (Eliquis)	5 mg BID	Reduce to 2.5 mg BID if ≥ 2 criteria are met: • Age ≥ 80 years • Weight ≤ 60 kg • Serum creatinine ≥ 1.5 mg/dL
Rivaroxaban (Xarelto)	20 mg daily (with food)	Reduce to 15 mg daily if CrCl is 15–50 mL/min. Hold if CrCl < 15 mL/min.
Dabigatran (Pradaxa)	150 mg BID	Reduce to 75 mg BID if CrCl is 15–30 mL/min. Avoid if CrCl < 15 mL/min.
Edoxaban (Savaysa)	60 mg daily	Reduce to 30 mg daily if CrCl is 15–50 mL/min (US NVAF label). Avoid if CrCl >95 or <15 mL/min. Weight ≤60 kg alone is a US VTE, not NVAF, dose-reduction criterion.

ELAN Trial: Fischer U et al. *N Engl J Med.* 2023;388:2411-2421. [PMID: 37222476](#)
CATALYST Meta-Analysis: Dehbi HM et al. [Lancet 2025; PMID: 40570866](#). Pooled data (n=5,441) from [ELAN \(PMID: 37222476\)](#), [OPTIMAS \(Lancet 2024; PMID: 39491870\)](#), [TIMING \(Circulation 2022; PMID: 36065821\)](#), and [START \(JAMA Neurol 2025; PMID: 40163159\)](#). Early DOAC (median Day 2) vs delayed (median Day 7-8) showed no excess sICH (0.4% vs 0.4%) and fewer recurrent ischemic events in pooled data.
US dosing label: [Savaysa §2.1 \(NVAF\)](#); check each current label for interactions and renal dosing.
AFib Guidelines: Joglar JA et al. 2023 ACC/AHA/ACCP/HRS Guideline. *Circulation.* 2024;149:e1-e156. [PMID: 38033089](#)